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Global Lifecare

PROVIDER COST EXCELLENCE

Proposition document for VPS Lakeshore Hospital

April-2026

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OUR PROPOSITION FOR HOLISTIC COST EXCELLENCE AT VPS LAKESHORE

1	VPS Lakeshore Hospitals is one of the largest and most trusted hospitals in Kochi demonstrating **robust revenue growth over the last 3 years** driven by patient experience and a comprehensive clinical program.
2	However, **margin expansion remains constrained** due to rising material costs (pharma, consumables, implants), increasing doctor payouts, and operational inefficiencies across clinical and administrative processes.
3	Based on our experience, there is a **significant headroom for cost take out** through a structured transformation program focused on eliminating inefficiencies in people productivity and material costs
4	Our belief comes from having **served 10+ providers over the last 5 years**, where we have consistently delivered savings typically in the range of 7-12% of addressable spend, driven through procurement, and operational interventions
5	We bring a **robust toolkit**, spanning across all key spend areas of a typical multispecialty hospital i.e., Material cost (Pharmacy, consumables, implants), People (Doctors, nursing, admin) and Indirect costs (Facilities, IT, Utilities)
6	We integrate technology into our ways of working by leveraging **advanced analytics, benchmarking databases, and digital tools** to enable rapid opportunity identification and data-backed decision making
7	We work closely with cross-functional hospital teams (clinical, procurement, finance, operations) in a one team model, ensuring alignment and ownership. We are committed to outcome delivery and are open to aligning incentives with impact
8	We are excited about the opportunity to serve you and **willing to invest in a 3-4 weeks diagnostic focused on material cost optimization that can help us jointly validate the size and source of opportunity and build confidence on the success of the exercise**

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Non-Exhaustive | Illustrative

OUR TEAM HAS WORKED ACROSS 10+ PROVIDERS OVER THE LAST ~5 YEARS ON THE TOPIC OF COST EXCELLENCE

	Case	Spend Base	Impact (INR, %)
1	Procurement transformation across pharmacy & consumables, including MAP-based pricing, supplier renegotiation, and formulary rationalization for multi-location hospital chain	Materials (Pharma and Consumables)	~INR 16-18 Cr, ~22%
2	Procurement transformation for a large hospital in South India with standardization, formulary restructuring, and GPO-led sourcing across inpatient and outpatient supplies	Materials (Pharma and Consumables)	~INR 9-10 Cr
3	Administrative cost transformation across 15+ functions including diagnostic assessment, zero-based redesign, and development of 3-year operating plans	Manpower & Indirect (Admin)	~INR 56 Cr
4	Peri-operative cost transformation including operating model assessment, opportunity prioritization, and roadmap development across governance, staffing and procurement	Clinical & Manpower costs	~6% of addressed spend
5	Clinical platform cost transformation for a large IDN, including cost assessment, roadmap development, and program governance enablement	Clinical & Admin costs	4-6% of addressed spend
6	Supply chain transformation for facilities management enterprise, including pricing assessment, GPO negotiation strategy, and identification of procurement and distribution saving opportunities	Materials & Services costs	6-8% of addressed spend

WE HAVE DELIVERED IMPACT FOR 90%+ OF THESE TOP PROVIDERS

Aster Hospital	MAX Healthcare	Apollo Hospitals
Medanta	Fortis Hospitals	Manipal Hospitals
AMRI Hospitals	Meitra Hospital	KIMS Hospitals
Paras Health	Care Hospitals	Sahyadri Hospitals
ProHealth Care	NorthShore University HealthSystem	Sodexo

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1. COMPREHENSIVE PROCUREMENT TRANSFORMATION TO DRIVE INR 16-18 CR SAVINGS (~22%) FOR A MULTI-LOCATION HOSPITAL CHAIN

INR 16-18 CR total opportunity identified on ~INR 78 Cr spend base (~22% impact)

Client situation:

- **~INR 98 Cr.** Annual spend across pharmacy (60 Cr.) and consumables (38 Cr.)
- **~80%** spend (~INR 78 Cr.) identified as focus scope for optimization
- **~3,625 SKUs** in Pharma and Consumables
- **Suboptimal procurement practices:** Limited price benchmarking vs market; high dependence on incumbent brands; inconsistent supplier negotiation outcomes
- **Weak system enablement:** Manual compliance tracking; limited analytics and automated reporting

Approach:

- **Top-down opportunity sizing+ bottom-up execution:** Defined Maximum Allowable Price (MAP) using market intelligence (tenders, imports, peer benchmarks)
- **Dual-path savings strategy to maximize impact**
  - Same brand optimization with renegotiation of existing supplier contracts
  - Substitution & consolidation with shift to clinically equivalent, cost-effective brands and rationalized formulary
- **Wave-based execution across full spend base**
  - Category-wise deep dives across pharmacy and consumables
  - Multi-wave RFQ rollout and supplier negotiations covering entire focus spend
  - Prioritized high-impact categories and suppliers for rapid value capture

Impact:

INR 16-18 Cr **savings identified**

INR 10-11 Cr opportunity in **pharmacy**

INR 6-7 Cr opportunity in **consumables**

65% opportunity executable with **low-moderate clinical intervention**

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2. PROCUREMENT TRANSFORMATION OF A LARGE HOSPITAL IN SOUTH INDIA

INR 9-10 CR total opportunity identified on ~INR 65 Cr. spend base

Client situation:

- **~INR 65-66 Cr** annual spend across pharmacy (42 Cr) and consumables (23 Cr)
- **~2,400+ SKUs** including retail pharmacy and OTC products
- **Suboptimal margins** due to lack of benchmarking; material cost ~24.5% of revenue
- **Low formulary compliance (~75%)** with multiple brands and price variability
- Limited visibility on non-formulary usage and weak approval controls

Approach:

- **Data standardization:** Cleaning up of masters to create a standard item master for pharma and consumables
- **Restructuring the formulary:** Multidisciplinary team meetings to reduce the number of brands used and arrive at a revised formulary
- **Process reengineering:** Lack of visibility of brands other than formulary items in the HIS. Approval mechanism for non-formulary items
- **Multipronged approach to solutioning:**
  - Inpatient supplies outsourced with negotiations to a GPO
  - Outpatient supplier: introduction of private labels
  - OTC: Limited optional based on consumption pattern

Impact:

INR ~8 Cr realized from inpatient supplies optimization

90% formulary compliance achieved

~400 bps margin improvement

21.5% of revenue as material cost

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3. ADMINISTRATIVE COST TRANSFORMATION TO DRIVE ~INR 56 CR SAVINGS AND ENABLE OPERATING MODEL OPTIMIZATION FOR A REGIONAL HOSPITAL SYSTEM

~INR 56 CR total opportunity identified over 3 years across labor and non-labor spend (admin functions)

Client situation:

- **Large regional healthcare** system with 3 hospitals and 12 clinics
- Significant margin pressure (~3.3%) with rising administrative cost base
- 15+ admin functions operating in silos (HR, Finance, IT, Materials)
- Limited visibility on FTE productivity, role duplication, true cost-to-serve per function
- Fragmented processes and non-standard operating model across facilities
- Lack of structured performance tracking and accountability

Approach:

- **Top-down diagnostic & bottom-up validation** with functional deep dives across 15+ admin areas and activity mapping, stakeholder interviews, FTE benchmarking
- **Zero-based organization (ZBO) redesign:**
  - Rebuilt functions from ground up
  - Eliminated non-value adding activities
  - Identified duplication and span inefficiencies
- **Voice of customer integration** by capturing enterprise-wide feedback on admin effectiveness
- **Operating model transformation** by standardization & centralization of key functions; defining future-state organization structure and governance
- Developed 3-year operating plans and FTE optimization database

Impact:

~INR 56 Cr savings identified over 3 years

32 FTEs rationalized through role redesign and efficiency improvements

Standardized processes across facilities reducing duplication

Improved accountability via defined ownership and governance

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1 Our toolkit for holistic provider cost excellence

2 Proposed next steps

3 Appendix: Our credentials on the topic

WE HAVE ROAD-TESTED PROVIDER COST EXCELLENCE TOOLKIT PROVEN ACROSS MULTIPLE TRANSFORMATION PROGRAMS

A MATERIAL COSTS	B INDIRECT COSTS	OLIVER WYMAN HOLISTIC PROVIDER COST EXCELLENCE	C DOCTOR COSTS	D PEOPLE COSTS
Benchmarking driven negotiation	Utilities cost optimization		Productivity benchmarking	Productivity & workload optimization
Consolidation and alternate sourcing	Service levels & contract optimization		Optimization of revenue per doctor	Process simplification / de-duplication
Consumption management	Consumption management		Payout re-balancing (e.g., Fixed vs variable)	
De-specification				

E DIGITAL ENABLEMENT

- Benchmarking tools e.g., Price benchmark, cost sheet etc.
- Best-practice enablers e.g., AA enabled tools to leverage internal / industry insights on consumption
- Process automation engines e.g., Automated contract review etc.

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Illustrative for a 1000 Cr multi-specialty hospital

ON A ~INR 1000 CR MULTI SPECIALTY HOSPITAL SPEND BASE, WE TYPICALLY OBSERVE INR 55-80 CR SAVINGS FROM A COST-EXCELLENCE PROGRAM

All figures in INR CR

	Cost Header	Description	FY25 Spend base (INR Cr)	Addressable Spend (%)	Addressable Spend base (INR Cr)	Typical Savings (%)	Potential opportunity (INR Cr)
A	Material cost	Pharmacy Medicines, Hospital consumables etc	280-320	~70%	200-225	8-10%	15-25
B	Employee cost	Cost of nursing staff, corporate staff, fixed payout to doctors etc	280-320	~80%	225-250	10-12%	25-30
C	Doctor fees	Revenue share paid to doctors	190-230	~40%	75-90	8-10%	5-10
D	Other expenses	Canteen expenses, house keeping etc	180-210	~80%	145-165	5-10%	10-15
	Total		950-1050		645-730		55-80

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Non-exhaustive

A. MATERIAL COST: WE LEVERAGE AN EXTENSIVE SET OF LEVERS TO OPTIMIZE MATERIAL COSTS ACROSS PHARMACY, CONSUMABLES AND IMPLANTS

Deep-dive ahead

	THEME	LEVERS	
MATERIALS COSTS	PHARMACY	1	Negotiation with KAMs of Indian and MNC pharma cos by leveraging
		A	Price Benchmarking Engine to arrive at "Target Cost/Margin" through competitor benchmarking
		B	Consolidation of volumes across companies/ brands to secure volume discount
		C	Strategic partnerships / white labelling for high volume products to enhance margins
		D	ZBC (Zero based costing tool) to identify "should cost" for high value drugs
		2	Alternate vendor threat for low margin-high volume products with high margin substitutes
	CONSUMABLES (INCL. LAB REAGENTS)	3	De-specification of non-critical consumables e.g., Transition to non-sterile exam gloves in OPD
		4	Harmonization of consumable specifications across departments e.g., Gloves type and sizes
		5	Consolidation of volumes across companies/ brands to secure volume discount
		6	Standardization of consumable consumptions across medical and surgical procedures to optimize consumptions
	IMPLANTS	7	Consolidation of volumes across companies/ brands to secure volume discount
		8	Strategic partnerships / white labelling for high volume products to enhance margins
		9	Optimization of contract terms and conditions to minimize leakages and improve margins

DIGITAL SOLUTIONS

Price Benchmarking Engine
Proprietary inhouse product benchmarks curated leveraging expert inputs to unlock procurement value for hospitals

Consumption optimizer
DnA enabled repository with standardized bill of material for 100+ medical and surgical procedures

Tail spend optimizer (TSO)
ML-powered product name grouping to reveal consolidation and sourcing opportunities

ZBC: Zero based costing tool
Combine platform analytics with expert judgment to model medicine, consumables and implant costs.

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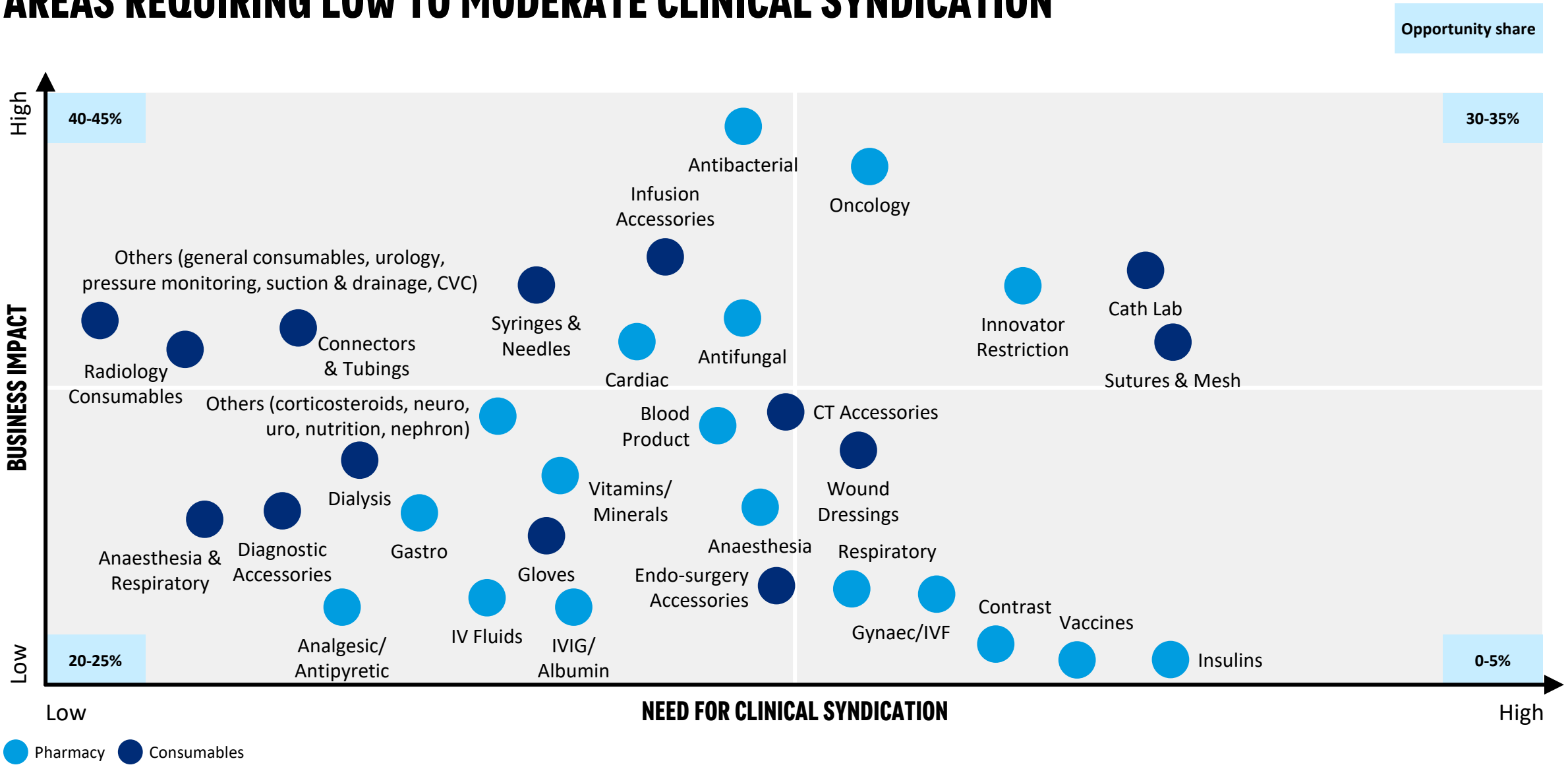
Illustrative

A.1,2. MATERIAL COST: IN OUR EXPERIENCE 60-70% OPPORTUNITY CAN BE UNLOCKED FROM AREAS REQUIRING LOW TO MODERATE CLINICAL SYNDICATION



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Illustrative

A.1,2. MATERIAL COST: THIS IMPROVEMENT IS TYPICALLY ACHIEVED ACROSS KEY METRICS WITHOUT SIGNIFICANT CHANGE IN SPEND PROFILE

	Parameter/KPI	As-is	Proposed	Best-in-class
PHARMACY	1 Gross Margin %	59% Top 70%: 61% Tail spend: 44%	65% Top 70%: 69% Tail spend: 44%	64-65%
PHARMACY	2 Brands per molecule Injections	2.1	1.7	1.3 – 1.4
PHARMACY	3 Business share by supplier type, %	Others 29%, MNC 36%, Top Indian 35%	Others 29%, MNC 37%, Top Indian 34%	Others 25-30%, MNC 25-30%, Top Indian 40-45%
PHARMACY	4 Average margin by supplier type, %	46%, 68%, 65%	55%, 76%, 71%	50-55%, 65-70%, 65-70%
PHARMACY	5 Innovator Share, %	9%	7%	8-10%
CONSUMABLES	1 Overall Margin	71% Top 70%: 73% Tail spend: 58%	74% Top 70%: 78% Tail spend: 58%	74-75%
CONSUMABLES	2 MNC Share	MNC 67%, Indian 33%	MNC 57%, Indian 43%	MNC 50%, Indian 50%

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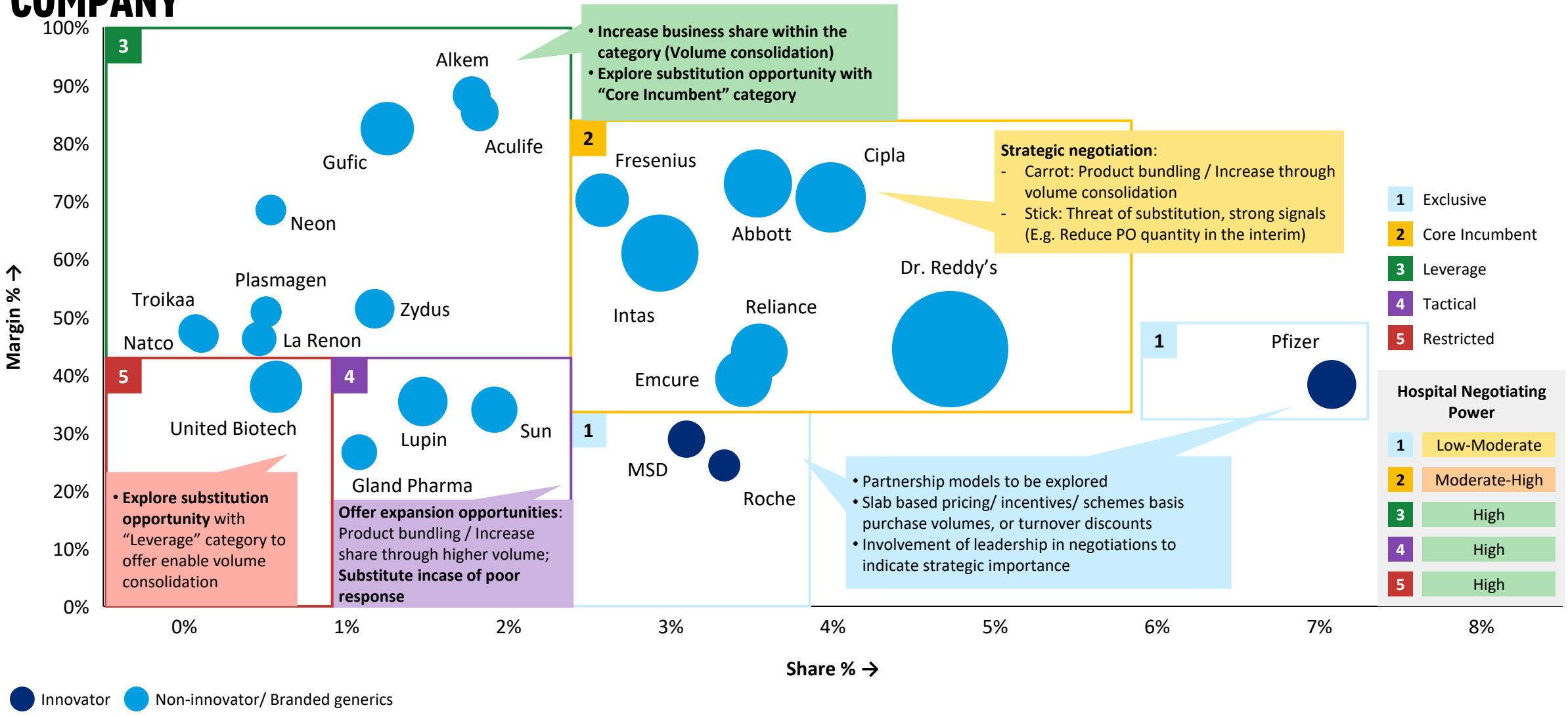
Client example

A.1,2. MATERIAL COST (PHARMACY): A DIFFERENTIATED NEGOTIATION STRATEGY IS DEPLOYED DEPENDING ON PRODUCT, SUPPLIER AND CURRENT POSITIONING OF THE COMPANY



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Sanitized client example

A.1,2. MATERIAL COST (PHARMACY): WE LEVERAGED OUR PROPRIETARY PRICE BENCHMARKING ENGINE FOR TRASTUZUMAB 440MG INJECTION (ONCOLOGY) TO IMPROVE MARGIN BY 20 P.P.

Annual Volumes	452 units
Annual Spend	80.4 Lakhs
Current Margin	48%

Consumption Type	Annual Volumes	Current Weighted Average	Proposed Weighted Average
Schemes	96	Cost – 16,454	Cost – 9,100
Cost Group	6	Cost – 48,770	Cost – 9,100
Margin Group	350	Margin – 17,764	Margin – 22,731

Opportunity	Cost: **24 lakhs** ⇩ Margin: **47 lakhs** ⇧
Proposed Margin	**68%** 20 P.P. ⇧

RFQ triggered to existing & 4 new players identified via benchmarking engine

Company	Cost	MRP	Brand Share	Benchmark	RFQ
Intas	14,282	20,481	46%	11,000	12,191
Zydus	15,000	40,754	19%	11,500	12,000
Dr. Reddy's	23,000	40,425	16%	20,000	23,000
RPG	14,000	58,898	6%	12,500	14,000
Roche	48,770	63,233	5%	45,000	*No response*
Celon	13,750	30,000	5%	12,500	12,738
Glenmark	18,000	67,535	4%	13,000	14,000
Hetero	-	51,579	-	**9,000**	9,100
Alkem	-	51,246	-	**10,000**	10,500

… and proposed volume consolidation across vendors to improve the margin %

Company	Group	Cost price	MRP	% Share
Hetero	S / M / C	9,100	51,759	~40%
Alkem	M	10,500	51,246	~15%
Intas	M	12,191	20,481	~40%
Roche	M (Innovator)	48,770	63,233	~5%

Alternate RFQ Quotes received:
Biocon* – 11,000 (Benchmark – 11,000) | Lupin – 14,000 (Benchmark – 13,000)

C – Cost Group; M – Margin Group; S – Schemes; MAP – Maximum Allowable Price

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p.p. = percentage points
*Due to supply challenges in the past, Biocon was not proposed for consolidation

Sanitized client example

A.5. MATERIAL COST (CONSUMABLES): WE UNLOCKED ~40% VALUE DRIVING VENDOR CONSOLIDATION IN BALLOON CATHETERS FOR A MULTI SPECIALITY HOSPITAL

PRODUCT DETAILS

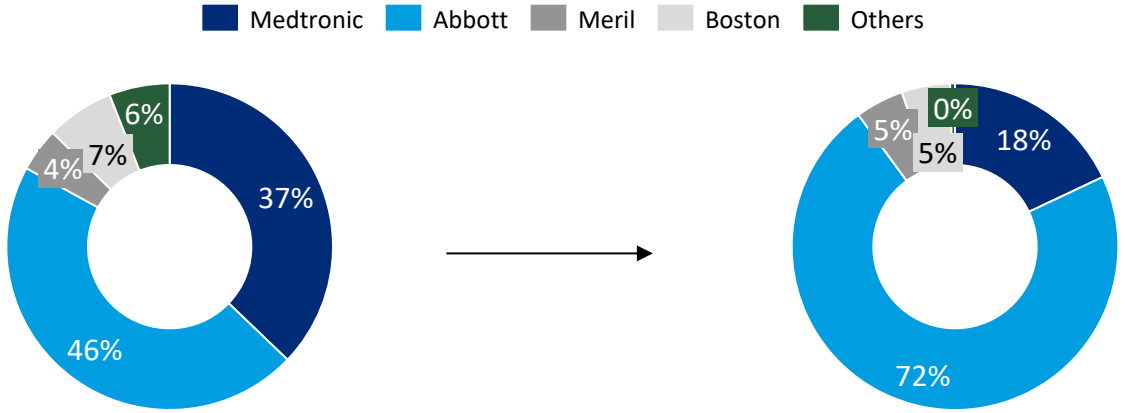
Balloon Catheters

1. **COMMERCIAL EFFECTIVENESS: HIGH**
2. **BRAND MIX: HIGH**
3. **SPEND: INR 73 LAKH**
4. **OPPORTUNITY: INR 29 LAKH (40%)**

- Negotiation: **INR 19 LAKH (26%)**
- Optimization of brand mix: **INR 10 LAKH (14%)**

KEY LEVERS

Transition volumes to low cost but reputed brands and consolidate volumes to 4 brands



Company	Spend INR lakh	Annual vol (Units)	Current rate INR/Unit	Target rate INR / Unit	Negotiation INR Lakh	New brand mix (Units)	Savings new mix (INR Lakh)
Medtronic	31	310	10,000	7,500	**8**	150	**5**
Abbott	25	382	6,550	4,500	**8**	600	**-**
Meril	4	36	11,000	7,500	**1**	40	**(0)**
Boston	6	56	10,714	7,500	**2**	40	**0**
Others	7	50	14,000	14,000		4	**4**
Total	73	834			**19**	834	**10**

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Sanitized client example

A.6. MATERIAL COST (CONSUMABLES): WE USE ADVANCED TEXT MINING TECHNIQUES WERE USED TO CLEAN AND REFINE PROCUREMENT DATA TO DELIVER 12-13% SAVINGS

SITUATION:

A hospital with highly fragmented repairs and consumables purchase

No unique material codes, leading to inability to consolidate vendors, spot price arbitrage losses, and get volume discounts

~3500-line items with ~2500 item codes

Example: **Gloves — 90 items**

S. No	Short Text	Cost per unit	Quantity	Amount
1	Latex Glvs 16"	20	40,102	802,040
2	Latex Gloves 18"	20	44,474	889,480
3	Latex Glove 14"	20	20,912	418,240
4	Powder free Latex Gloves 12"	20	25,352	507,040
5	Latex Gloves Free Size	21	29,254	614,334
6	Gloves Latex 18"	19	34,280	651,320
7	Latex Glvs 12"	20	21,885	437,700
8	Glvs Latex 14"	22	27,049	595,078
9	Glvs 14"	20	34,342	686,840
10	Glvs 18"	20	31,696	633,920
11	Glvs 12"	22	48,837	1,074,414
12	Glvs 16"	19	23,322	443,118
13	Glvs Free	21	17,323	363,783
14	Gloves Latex 14"	20	24,091	481,820
15	Gloves Latex 16"	20	29,946	598,920

APPROACH FOR TEXT MINING:

1	**Natural Language Processing**	Advanced NLP to extract meaningful features from free text
2	**Fuzzy Matching**	Eliminated spelling mistakes (e.g., Glove, Gloves, Glv; all mapped to "Gloves")
3	**Advanced Clustering techniques**	Clustering within a category (e.g., Sterile Gloves – Latex, Nitrile Gloves, Examination Gloves) using algorithms like LDA, semantic clustering, etc.
4	**Merging with Business Knowledge**	Merged clusters with business sense to tag clusters with meaningful labels

CATEGORIZATION:

Grouped gloves into 5 categories automatically

- Nonsterile – Powder free
- Sterile – Gamma radiated
- Sterile - Latex
- Examination gloves
- Nitrile

IMPACT OBSERVED:

12–13% savings identified across repairs and consumables

PO classified into key sub-categories:

- ~3500-line items mapped into ~25 categories

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B,C. PEOPLE COST: WE HAVE AN ESTABLISHED SET OF LEVERS TO OPTIMIZE PEOPLE COST IN PROVIDERS

Deep-dive ahead

SPEND BASE	LEVERS	
B. EMPLOYEE COST	1	Productivity improvement by de-duplication of tasks, elimination of non-value adding tasks, simplification of necessary-non-value adding tasks, and deploying digital system to eliminate time consuming manual tasks
	2	Spans and layers optimization opportunity across functions and roles to right size organization
C. DOCTOR FEES	1	Optimization of doctor payout model between retainer, fixed + variable, and variable only depending on department, revenue scale and place of operations
	2	Drive productivity improvement for doctors through simplification / elimination of non-value add administrative processes enabling doctors with higher patient time e.g., leveraging junior doctors & nurses in OPD for vaccinations, prescription writing etc.

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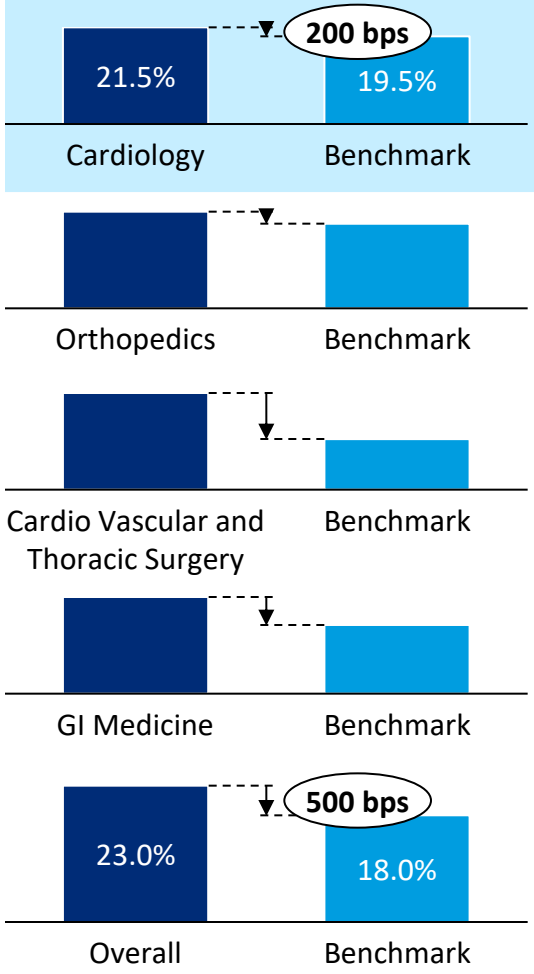
C.1. PEER-BENCHMARKS INDICATED AN OPPORTUNITY TO REDUCE DOCTOR PAYOUT FROM 23% TO 18% OF REVENUE; PILOT IMPLEMENTATION IN CARDIOLOGY DEMONSTRATED 150 BPS IMPROVEMENT

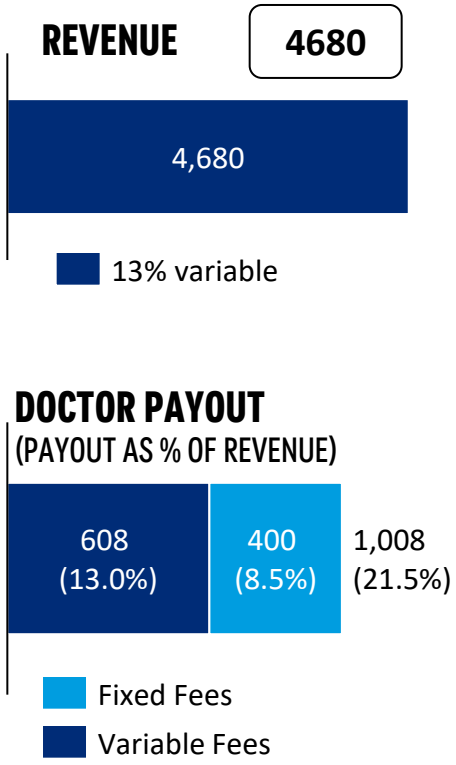
All figures in INR Lakh

DOCTOR PAYOUT/ REVENUE BENCHMARKS BY DEPARTMENT



CARDIOLOGY DEPARTMENT

FY25 – BEFORE PAYOUT RESTRUCTURING



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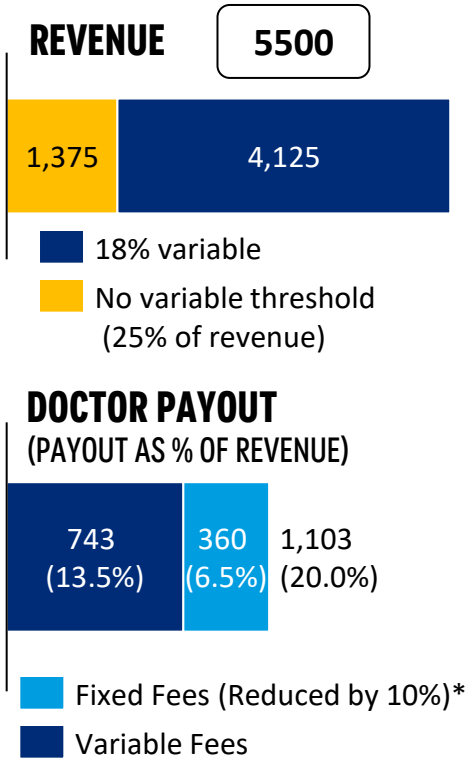
FY26 - AFTER PAYOUT RESTRUCTURING



IMPACT
(WIN-WIN FOR DOCTORS AND HOSPITAL)

18% Increase in revenue because of increase in variable fee payout for doctors from 13% to 20% of revenue

150 BPS Reduction in doctor payout as a % of revenue

Introduction of "payout threshold" secures the hospital margins against future dips in performance (If any)

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*Savings on fixed payout would compound year on year as cost savings for the hospital

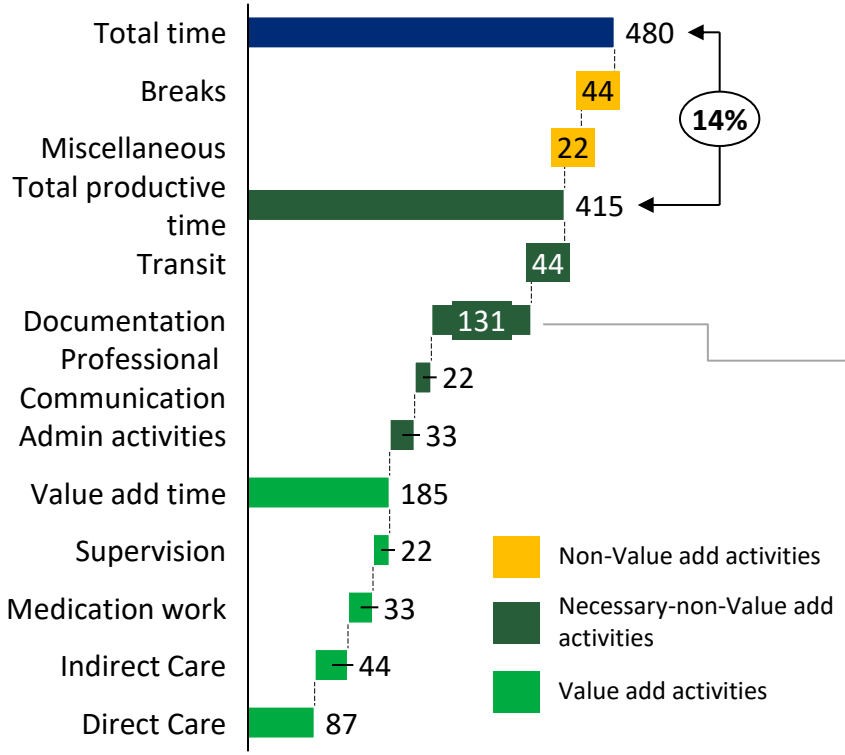
Illustrative

B.1. PEOPLE COST (EMPLOYEE): WE IDENTIFIED OPPORTUNITY TO IMPROVE NURSE PRODUCTIVITY BY ~15%

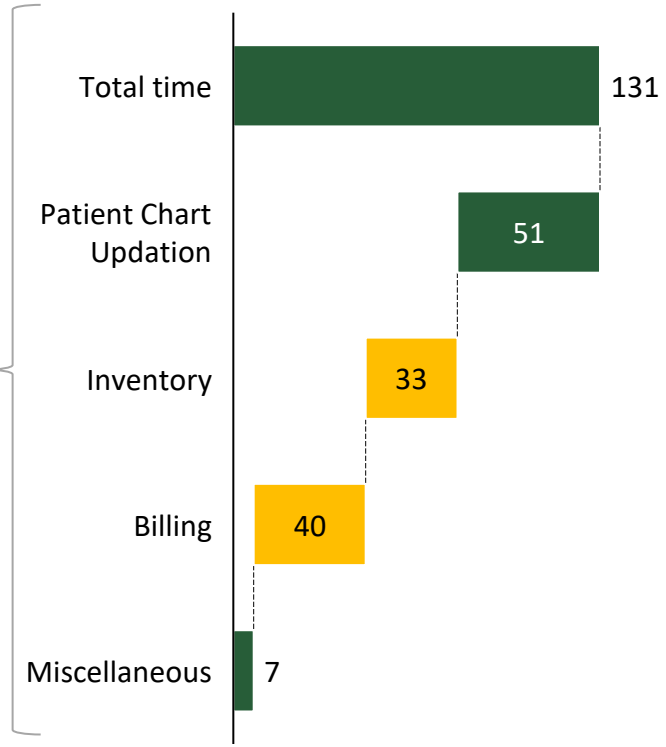
Basis "DILO" for a 8 hour shift of a nurse in General ward

NURSE X IN GENERAL WARD

Time in minutes



DOCUMENTATION COMPRISES OF 27% OF THE TOTAL TIME SPENT IN ONE SHIFT



Root cause

- Duplicate data entry in patient charts between doctor notes and nurses
- Multiple log-books recording same inventory details
- Manual vital recording for patients

Proposed solution

- **Electronic patient medical records (EMR)** to eliminate duplicate data entry
- Automated inventory management and billing using the EMR data as input
- Vital recording to be digitally tracked from ECG machines

IMPACT : Activities consuming 73 mins (**15% of total time**) can be eliminated leveraging EMR for patient carts.
Pursuing similar low cost automation interventions across all areas enabled reduction in nurse per bed from 2.5 to 2

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D. OTHER EXPENSES: WE LEVERAGE EXTENSIVE SET OF LEVERS TO OPTIMIZE INDIRECT COSTS (1/2)

SPEND BASE	LEVERS	
A. ADMIN EXPENSES (INCLUDING MAINTENANCE & REPAIRS)	1	Consolidate facility management spends e.g., Housekeeping, security etc
	2	Negotiation of service charges across Security, Laundry and Housekeeping services
	3	Optimization of canteen and pantry expenses e.g., Negotiation with exiting vendors / alternate vendors
	4	CMC vs AMC & Insurance combination of high-cost critical equipment & paid visit for low-cost equipment
	5	Optimization of Insurance spend e.g., Capped insurance cover for assets
	6	Elimination of IPD stationary spend: Transitioning from printed charts / records to digital monitoring
B. IT EXPENSES	1	IT spend optimization e.g., moving from proprietary exchange to open-source systems, rationalization of licenses, optimization of bandwidth requirement etc
C. TRAVEL AND HOSPITALITY	1	Policy levers: Benchmark travel policy for doctors and corporate staff with industry best practices and recommend changes (e.g., advance booking, etc.)
	2	Rate levers: Negotiate rates by onboarding additional vendors, airlines, hotels, car, as a part of corporate travel or alternatively outsource services to 3rd party for cost effective management
	3	Consumption levers: Transition to virtual events from physical in-person meetings / events etc
D. CORPORATE FUNCTIONS	1	Spans and layers optimization opportunity across functions and roles to right size organization
	2	Productivity improvement by elimination duplication, simplification of process, etc.
	3	Strategic outsourcing/ offshoring of services

DIGITAL SOLUTIONS

Contracts AI

- GenAI scans 1,000+ contracts to surface rapid improvement opportunities

Tail spend optimizer (TSO)

- ML-powered product name grouping to reveal consolidation and sourcing opportunities

ODL: Organization design laboratory

- Optimize spans, layers & lean structure

Utility Optimizer

- End-to-end hotspot detection, root-cause analysis and targeted interventions for utilities

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D. OTHER EXPENSES: WE LEVERAGE EXTENSIVE SET OF LEVERS TO OPTIMIZE INDIRECT COSTS (2/2)

Deep-dive ahead

SPEND BASE	LEVERS		DIGITAL SOLUTIONS
E. POWER & FUEL	1	**Consumption & transmission levers**: - Consumption optimization e.g., Installation of motion sensors, Installation of building management system to regulate HVAC temperature etc. - Reduction of transmission losses e.g., Unit power factor, Improving transformer efficiency, identification and minimizing gas leakages like N2, O2 etc	***Contracts AI*** *- GenAI scans 1,000+ contracts to surface rapid improvement opportunities* ***Tail spend optimizer (TSO)*** *- ML-powered product name grouping to reveal consolidation and sourcing opportunities* ***ODL: Organization design laboratory*** *- Optimize spans, layers & lean structure* ***Utility Optimizer*** *- End-to-end hotspot detection, root-cause analysis and targeted interventions for utilities*
	2	**Efficiency levers**: - Replacement of heavy duty ageing equipment with low cost alternatives e.g., Use of BLDC motors in HVAC instead of traditional AC motors, IE1/2 motors to IL3+ motors - Use of efficiency improvement equipment for utility heavy machines e.g., ATCS in chillers, use of heat pump as autoclave inlet to reduce steam load, combining headers across chillers to reduce load, etc	
	3	**Generation Levers**: - Leveraging renewable energy to offset grid electricity e.g., Rooftop solar, Open access power purchase - Alternate fuel sources for specific equipment	

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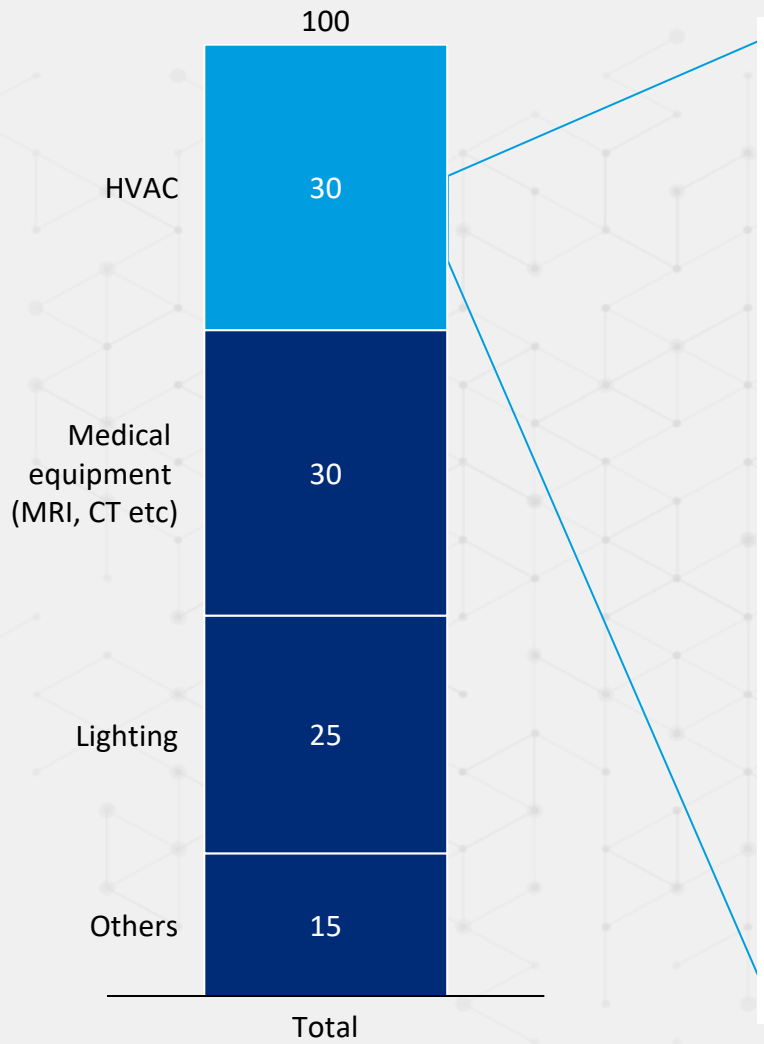
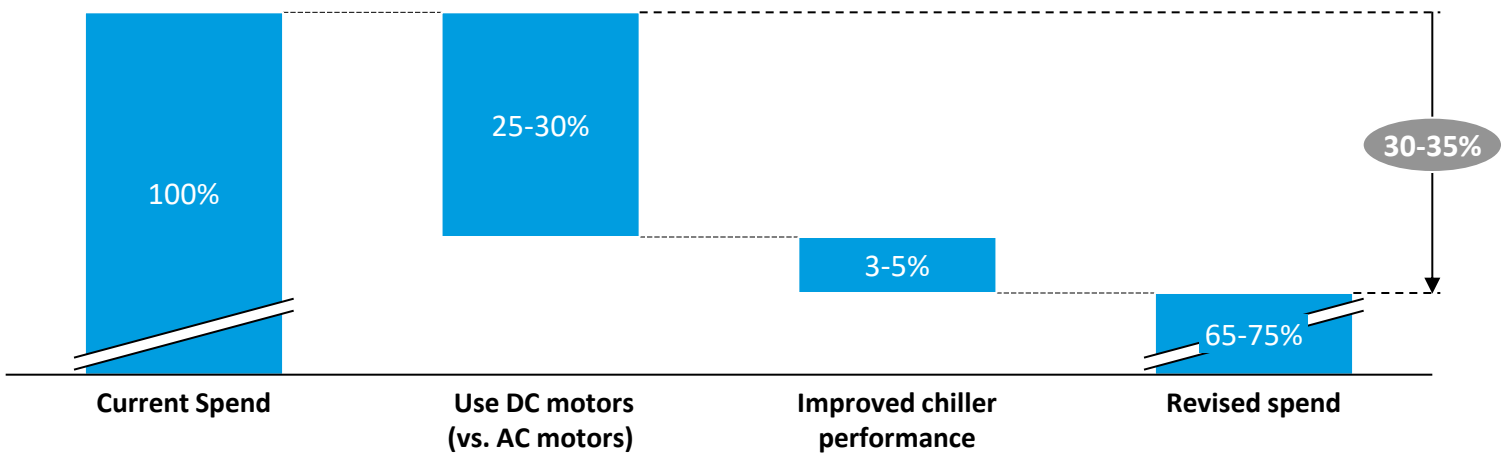
Sanitized client example

D. OTHER EXPENSES - UTILITIES: 30-35% REDUCTION IN HVAC SPEND IN 1 YEAR OF IMPLEMENTATION



30-35% reduction in HVAC power consumption



- Energy loss minimized due to direct drive
- Automatic speed variation depending on area requirement
- Higher reliability, life expectancy and lower maintenance

- Reduction in heat load due to DC motor
- Auto-tube cleaning system (ATCS) to improve chiller efficiency

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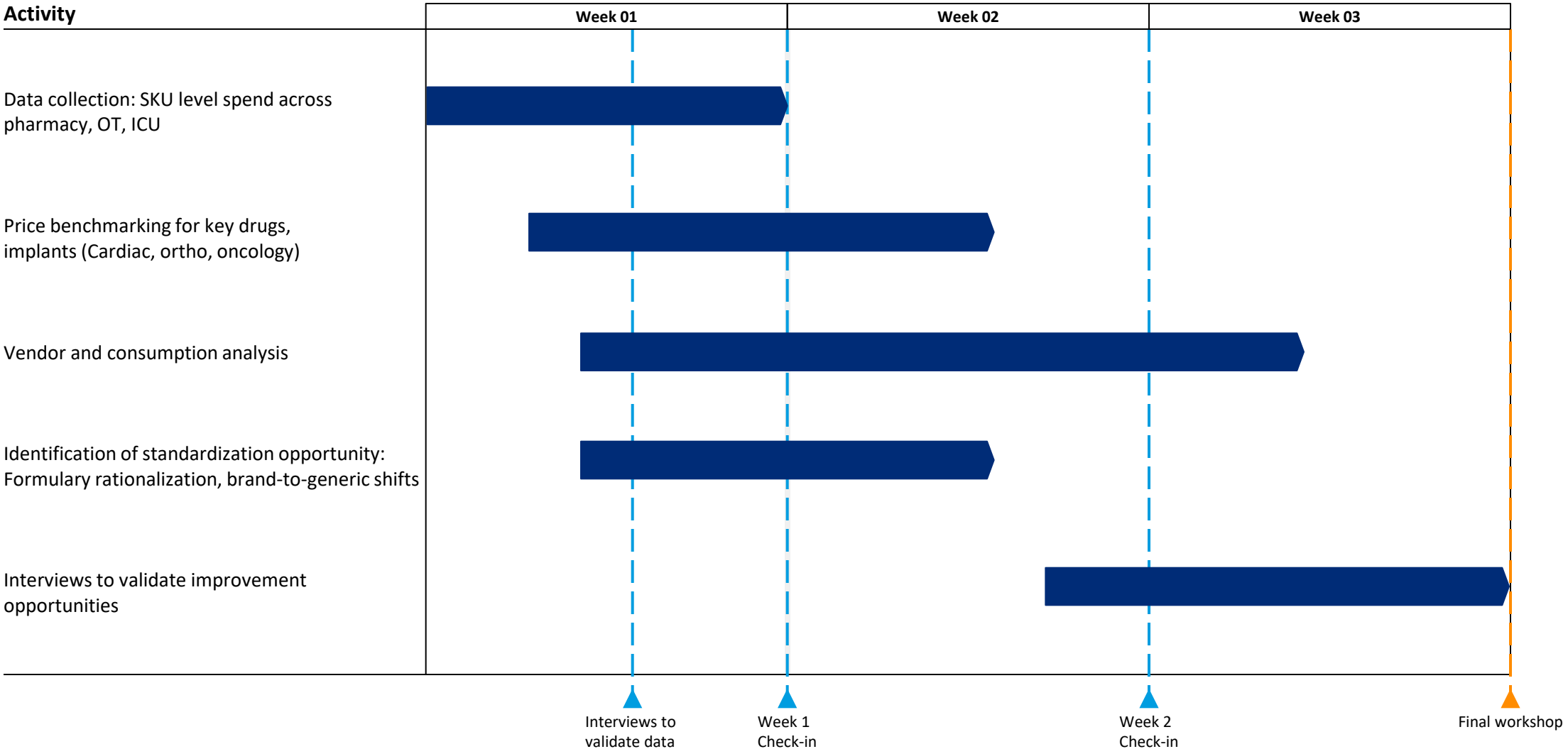
1 Our toolkit for holistic provider cost excellence

2 Proposed next steps

3 Appendix: Our credentials on the topic

WE CAN CONDUCT A STRUCTURED DIAGNOSTIC SPANNING ~3 WEEKS TO COME UP WITH A CLEAR ARTICULATION OF OPPORTUNITY AND DETAILED EXECUTION ROADMAP

MEDICINE, IMPLANTS AND OTHER DIRECT MATERIAL COST



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WE BELIEVE THAT OLIVER WYMAN IS UNIQUELY POSITIONED TO BE YOUR TRUSTED PARTNER IN THIS TRANSFORMATION

A

GLOBAL FIRM OPERATING ACROSS 30 COUNTRIES WITH EXPERTISE IN HOSPITALS

- Oliver Wyman is **global #3 in Healthcare consulting** with $4bn revenues (part of the Marsh group) and serving 50% of the top 500 global companies
- An experienced team of 100+ partners, principals and advisors drives our Healthcare practice

B

OUR LEADERSHIP TEAM WITH DEEP EXPERTISE IN PROVIDER TRANSFORMATIONS

- Our team of **partners, consultants and panel of senior advisors** have deep expertise in provider cost excellence
- Over the last ~5 years, our team has delivered 10+ programs of similar nature across providers

C

ROBUST TOOLKIT WITH PROPRIETARY DNA/AI SOLUTIONS

- We have a robust **toolkit with road-tested levers** on optimization of cost across spend categories i.e. material, employee, doctor fees and indirect spend
- This is supported by a holistic set of **10+ digital tools that enable faster and deeper impact**

D

STRONG COMMITMENT WITH ACTUAL SKIN IN THE GAME

- As a trusted partner, we are always willing to offer a variable fee option with our **fees directly linked** with impact
- Along with experience, our team also brings along **rigorous execution firepower** to ensure that the impact is delivered on time in full

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A. WE ARE PART OF MARSH – A US LISTED $100 BN MARKET CAP ENTITY – LARGEST PROFESSIONAL SERVICES GROUP GLOBALLY

MARSH

Annualized revenues: US$26.5 BN[1]

Staff: 90,000

Clients in more than 130 countries

New York Stock Exchange (MRSH)

OLIVER WYMAN
A MARSH BUSINESS

MARSH

GUY CARPENTER
A MARSH BUSINESS

MERCER
A MARSH BUSINESS

7,000+
Professionals worldwide

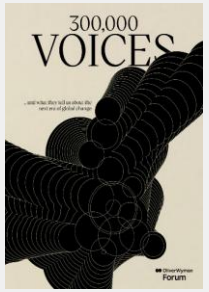
50%
Of the **top 500 global companies** are our clients

~$3.6 BN
Revenue annually[1]

70
Offices in **30 countries**

We are a global leader in management consulting

We represent the voice of industry and business across thought leadership and key global summit



Oliver Wyman at Davos 2026

Oliver Wyman India at Bio Asia 2026

1. Last 12 months ended March 31, 2026

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A. HLS IS OUR TOP 3 PRACTICE GLOBALLY WITH A TEAM OF 100+ PARTNERS, PRINCIPALS AND ADVISORS

Managing Partner

Chris Bernene

85 Americas Partners and Principals

Adam Harpool, Aditya Lingampalli, Aileen Rowan, Alok Dalal, Amanda Evison, Andrew Jakubowski, Andrew Kodesch, Ann Kaplan, Annika Tubito, Aron Cohen, Ashish Kaura, Ashley Smith, Ben Paik, Benjamin Rymzo, Cassie Burke, Charlie Hoban, Chris Schrader, Dan Hassing, Dan Shellenbarger, Deirdre Baggot, Derek Lukin, Doug Pfaff, Elizabeth Shakhnovich, Eric Lu, Greg Berger, Hailey Fowler, Heiyab Tessema, Helen Leis, Holly Noto, Howard Lapsley, Hugo Manassei, Igor Belokrinitsky, Ilina Sen, Iris Kim, Jamie Landsman, Jeremiah Lee, Jessica Rose, Jim Fields, John Caison, John Lester, Joshua Pagliaro, Julia Cormier, Julia Goldner, Kate Grady, Kevin Feng, Kevin Wistehuff, Laurie Bankhead, Lindsay Knable, Marc Tomassi, Marcia Macphearson, Marisa Flignor, Marisa Greenwald, Martin Graf, Mathieu Barthelemy, Michael Hill, Michael Muldoon, Minoo Javanmardian, Mofeed Sawan, Nick Karzmer, Nikhil Sarathi, Parie Garg, Patrick Barlow, Paul Mee, Peter Borden, Rachel Burstain, Rahul Ekbote, Ran Strul, Robert Pogulis, Rohith Banerjee, Russell Wong, Sam Winter, Santiago Doria, Sarah Chaudhry, Sarah Snider, Shivani Shah, Shruti Satish, Shyam Vichare, Sukanya Soderland, Terry Stone, Thomas Robinson, Timi Boyo, Tomas Mikuckis, Travis Kistler, Vamsi Kothapalli, Zhe Yu

15 Expert Advisors

Daphne Klausner, Kimberly Babbin, Nick Coussoule, Scott Ptacek, Stuart Shaw, Terry Burke, Dr. Mandar Vaidya, Dr. Jeremy Lim, Dr. Mahesh Singh, Vikas Bhadoria, Bart Jannsens, Dr. Ashok Bhatia, CS Muralidharan, Ashok Kumar, Samik Biswas

14 Europe Partners and Principals

Adrian Gibb, Andreas Eggert, Arjun Shyam, Clemens Freytag, Darshan Vora, Handre Schmidt, Jan Rys, Kamal Dhami, Klaus Bredl, Marc Muench, Marie-Lyn Horlacher, Oliver Eitelwein, Sourav Choudhury, Thibault Wautier

9 IMEA Partners and Principals

Adeel Khairi, Chirag Adatia, Mridul Karkara, Gautam Chawla, Partha Basumatary, Sumit Sharma, Samer Abi Chaker, Dr. Nitiraj Gandhi, Matt Stubbs

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Non-exhaustive

B. OUR LEADERSHIP TEAM HAS ONE OF THE DEEPEST EXPERIENCE OF WORKING WITH INDIAN & GLOBAL HOSPITALS ON OPERATIONS AND COST TRANSFORMATIONS

OUR INDIA HLS LEADERSHIP TEAM

Dr. Chirag Adatia
Senior Partner
McKinsey & Company
THB

Gautam Chawla
Partner
McKinsey & Company

Mridul Karkara
Partner
Monitor Deloitte.
THB

Dr. Nitiraj Gandhi
Director
Columbia Asia
EY Parthenon

OUR INDIA EXPERT TEAM

Dr. Mahesh Singh
Hospital Ops Lead
MAX Healthcare
Manipal Hospitals

Bart Janssens
Partner
BCG

Vikas Bhadoria
Sr Advisor
McKinsey & Company

OUR GLOBAL OW EXPERT TEAM

Tom Robinson
Partner

Sourav Choudhary
Partner

Sumit Sharma
Partner

Minoo Javanmardian
Partner

Dr. Jeremy Lin
Senior Advisor

INDIAN HOSPITALS SERVED ACROSS TRANSFORMATIONS

MAX Healthcare
Apollo Hospitals
Aster
Fortis
Medanta
CARE Hospitals
Narayana Health
Manipal Hospitals
Wockhardt
Dr Lal PathLabs
Metropolis
AMRI Hospitals

OUR GLOBAL CLIENTS

Cleveland Clinic
Mount Sinai
Kaiser Permanente
St Luke's
Mt. Grace Hospitals, Inc.
HCA Healthcare
Cornerstone Health Care
Metro Pacific Health — The Heart of Filipino Healthcare
UHS

The most experienced team in India hospitals | **3 Medical Doctors, 5 Operators, Exp of Consulting + Digital + Hospital Ops**

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Non-Exhaustive | Illustrative

B. OUR TEAM HAS WORKED ACROSS 10+ PROVIDERS OVER THE LAST ~5 YEARS ON THE TOPIC OF COST EXCELLENCE

	Case	Spend Base	Impact (INR, %)
1	Procurement transformation across pharmacy & consumables, including MAP-based pricing, supplier renegotiation, and formulary rationalization for multi-location hospital chain	Materials (Pharma and Consumables)	~INR 16-18 Cr, ~22%
2	Procurement transformation for a large hospital in South India with standardization, formulary restructuring, and GPO-led sourcing across inpatient and outpatient supplies	Materials (Pharma and Consumables)	~INR 9-10 Cr
3	Administrative cost transformation across 15+ functions including diagnostic assessment, zero-based redesign, and development of 3-year operating plans	Manpower & Indirect (Admin)	~INR 56 Cr
4	Peri-operative cost transformation including operating model assessment, opportunity prioritization, and roadmap development across governance, staffing and procurement	Clinical & Manpower costs	~6% of addressed spend
5	Clinical platform cost transformation for a large IDN, including cost assessment, roadmap development, and program governance enablement	Clinical & Admin costs	4-6% of addressed spend
6	Supply chain transformation for facilities management enterprise, including pricing assessment, GPO negotiation strategy, and identification of procurement and distribution saving opportunities	Materials & Services costs	6-8% of addressed spend

WE HAVE DELIVERED IMPACT FOR 90%+ OF THESE TOP PROVIDERS

Aster Hospital	MAX Healthcare	Apollo Hospitals
Medanta	Fortis Hospitals	Manipal Hospitals
AMRI Hospitals	Meitra Hospital	KIMS Hospitals
Paras Health	Care Hospitals	Sahyadri Hospitals
ProHealth Care	NorthShore University HealthSystem	Sodexo

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B.1. COMPREHENSIVE PROCUREMENT TRANSFORMATION TO DRIVE INR 16-18 CR SAVINGS (~22%) FOR A MULTI-LOCATION HOSPITAL CHAIN

INR 16-18 CR total opportunity identified on ~INR 78 Cr spend base (~22% impact)

Client situation:

- **~INR 98 Cr.** Annual spend across pharmacy (60 Cr.) and consumables (38 Cr.)
- **~80%** spend (~INR 78 Cr.) identified as focus scope for optimization
- **~3,625 SKUs** in Pharma and Consumables
- **Suboptimal procurement practices:** Limited price benchmarking vs market; high dependence on incumbent brands; inconsistent supplier negotiation outcomes
- **Weak system enablement:** Manual compliance tracking; limited analytics and automated reporting

Approach:

- **Top-down opportunity sizing+ bottom-up execution:** Defined Maximum Allowable Price (MAP) using market intelligence (tenders, imports, peer benchmarks)
- **Dual-path savings strategy to maximize impact**
  - Same brand optimization with renegotiation of existing supplier contracts
  - Substitution & consolidation with shift to clinically equivalent, cost-effective brands and rationalized formulary
- **Wave-based execution across full spend base**
  - Category-wise deep dives across pharmacy and consumables
  - Multi-wave RFQ rollout and supplier negotiations covering entire focus spend
  - Prioritized high-impact categories and suppliers for rapid value capture

Impact:

INR 16-18 Cr **savings identified**

INR 10-11 Cr opportunity in **pharmacy**

INR 6-7 Cr opportunity in **consumables**

65% opportunity executable with **low-moderate clinical intervention**

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B.2. PROCUREMENT TRANSFORMATION OF A LARGE HOSPITAL IN SOUTH INDIA

INR 9-10 CR total opportunity identified on ~INR 65 Cr. spend base

Client situation:

- **~INR 65-66 Cr** annual spend across pharmacy (42 Cr) and consumables (23 Cr)
- **~2,400+ SKUs** including retail pharmacy and OTC products
- **Suboptimal margins** due to lack of benchmarking; material cost ~24.5% of revenue
- **Low formulary compliance (~75%)** with multiple brands and price variability
- Limited visibility on non-formulary usage and weak approval controls

Approach:

- **Data standardization:** Cleaning up of masters to create a standard item master for pharma and consumables
- **Restructuring the formulary:** Multidisciplinary team meetings to reduce the number of brands used and arrive at a revised formulary
- **Process reengineering:** Lack of visibility of brands other than formulary items in the HIS. Approval mechanism for non-formulary items
- **Multipronged approach to solutioning:**
  - Inpatient supplies outsourced with negotiations to a GPO
  - Outpatient supplier: introduction of private labels
  - OTC: Limited optional based on consumption pattern

Impact:

INR ~8 Cr realized from inpatient supplies optimization

90% formulary compliance achieved

~400 bps margin improvement

21.5% of revenue as material cost

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B.3. ADMINISTRATIVE COST TRANSFORMATION TO DRIVE ~INR 56 CR SAVINGS AND ENABLE OPERATING MODEL OPTIMIZATION FOR A REGIONAL HOSPITAL SYSTEM

~INR 56 CR total opportunity identified over 3 years across labor and non-labor spend (admin functions)

Client situation:

- **Large regional healthcare** system with 3 hospitals and 12 clinics
- Significant margin pressure (~3.3%) with rising administrative cost base
- 15+ admin functions operating in silos (HR, Finance, IT, Materials)
- Limited visibility on FTE productivity, role duplication, true cost-to-serve per function
- Fragmented processes and non-standard operating model across facilities
- Lack of structured performance tracking and accountability

Approach:

- **Top-down diagnostic & bottom-up validation** with functional deep dives across 15+ admin areas and activity mapping, stakeholder interviews, FTE benchmarking
- **Zero-based organization (ZBO) redesign:**
  - Rebuilt functions from ground up
  - Eliminated non-value adding activities
  - Identified duplication and span inefficiencies
- **Voice of customer integration** by capturing enterprise-wide feedback on admin effectiveness
- **Operating model transformation** by standardization & centralization of key functions; defining future-state organization structure and governance
- Developed 3-year operating plans and FTE optimization database

Impact:

~INR 56 Cr savings identified over 3 years

32 FTEs rationalized through role redesign and efficiency improvements

Standardized processes across facilities reducing duplication

Improved accountability via defined ownership and governance

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B.4. PER-OPERATIVE COST TRANSFORMATION TO DRIVE 6% IMMEDIATE SAVINGS AND UPTO 12% OF BASELINE COST

For a community health system (Peri-operative function)

6% immediate opportunity identified, with up to **13%** of baseline cost through longer-term transformation

Client situation:

- **Community health system** in an affluent, competitive suburban geography
- Prior support focused only on inpatient service distribution
- Madate to transform peri-operative function at one-hospital
- Objective to build a regionally recognized orthopedic specialty facility
- Need to improve efficiency, cost structure, and competitive positioning

Approach:

- **Operating model assessment:** Evaluated current peri-operative workflows, staffing, and governance
- **Opportunity prioritization:**
  - Identified immediate vs long-term improvement levers
  - Focused on high-impact areas across peri-op value chain
- Sized opportunities across governance, staffing, scheduling, and procurement
- Brought in peri-op best practices and SME insights and defined future-state operating model and improvement roadmap

Impact:

6% immediate cost savings opportunity identified

Up to 13% of baseline cost achievable through longer-term initiatives

Identified improvement areas across:
- Governance
- Operating model
- Staffing & scheduling
- Implants & medical device procurement

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B.5. CLINICAL PLATFORM COST TRANSFORMATION TO DRIVE 4-6% SAVINGS AND ENABLE SUSTAINABLE MARGIN IMPROVEMENT

For a large integrated delivery network (~$30B system)

4-6% cost savings **(~$850M** run-rate reduction) identified alongside growth levers to improve membership trajectory

Client situation:

- **$30B** integrated health system facing margin and membership erosion
- Declining performance across key lines of business
- Operating in highly competitive insurance and provider markets
- **Margin trajectory turning negative within 2-3 years if unaddressed**
- Fragmented, locally-driven decision-making with high variability in operations

Approach:

- **Operations-forward cost transformation:**
  - Assessed cost structure across clinical platform
  - Identified system-wide cost optimization levers
- **Integrated roadmap development:**
  - Built single prioritized roadmap with quantified financial impact
  - Aligned cost reduction with growth and retention objectives
- **Market-back growth lens:**
  - Evaluated key growth and membership retention drivers
  - Linked operational improvements to member outcomes
- **Program governance enablement:**
  - Established central program management capability (PMO)
  - Deployed tools for visibility, tracking, and accountability

Impact:

4-6% **cost savings identified**

~1ppt improvement in 5-year membership CAGR

Created sustainable cost structure to fund strategic investments

Improved visibility & accountability via centralized governance

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B.6. SUPPLY CHAIN TRANSFORMATION TO ENABLE COST OPTIMIZATION AND COMPETITIVE REPOSITIONING

For a facilities management enterprise (Hospital segment)

Identified pricing, supply chain, and service optimization levers to unlock savings and improve competitiveness

Client situation:

- Large provider of on-site services and procurement solutions across corporates, schools, and hospitals with engagement focused on hospital segment
- Facing intense pricing pressure from Group Purchasing Organizations (GPOs)
- Fragmented approach across:
  - Pricing (Inefficient product pricing, especially food procurement)
  - Supply chain (Limited cost optimization levers)
  - Service offerings (Lack of differentiated positioning)

Approach:

- **Pricing transformation:**
  - Assessed product pricing structures and benchmarked vs market
  - Designed real-time negotiation strategy with multiple GPOs
- **Supply chain optimization:**
  - Identified savings levers across procurement, catalogue, and distribution
  - Created initiatives to offset price reductions through efficiency gains
- Defined new service offerings and value propositions
- Built supporting tools, databases, and implementation roadmap

Impact:

Defined **clear pricing strategy and negotiation playbook** with GPOs

Identified **6-8% savings opportunities in supply chain** to fund price competitiveness

Established **rebate-led value-sharing mechanisms** with clients

Created roadmap for new service offerings and segment-specific strategies

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Non-Exhaustive

C. WE HAVE A SET OF PROPRIETARY SUITE OF DNA/AI TOOLS AND SOLUTIONS TO MAXIMIZE IMPACT IN COST EXCELLENCE PROGRAMS

OW'S PROPRIETARY OFFERING	DESCRIPTION	IMPACT
1. Price Benchmarking engine	Proprietary inhouse product benchmarks curated leveraging expert inputs to unlock procurement value for hospitals	**10-15%** reduction in sourcing spend
2. Zero based costing tool	Digital parametric solution that combines extensive database on material costs, labor rates, etc. with expert-based inputs to develop should cost estimates of PM, FG etc.	**5-7%** spend reduction
3. Contracts.AI	GenAI powered contract evaluation tool to rapidly identify improvement opportunities in contracts and ensure compliance of invoicing and payments to contracts	**>90%** reduction in leakages
4. Utility Optimizer	An integrated end to end solution that conducts hot-spot identification of inefficiencies in utilities and conducts root cause assessment to propose interventions	**10-15%** reduction in utilities spend
5. Tail spend optimizer	Digital capability to learn from product names & categorize like to like products to potentially uncover opportunities across vendor consolidation, arbitrage, etc.	**5-7%** spend reduction
6. Organization design laboratory (ODL)	Proprietary diagnostic tool for organization design, spans and layers optimization and propose best in class lean org structure for operations	**10-30%** increase in productivity
7. Consumption Optimizer	AI enabled tool studies patterns of material consumption across different surgeons / procedures and determines best-in-class consumption practices to reduce wastages	**3-5%** wastage reduction

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